A Weekly Demand Briefing for Scotland's Emergency Departments

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AI Mastery Capstone: Industry Integrated AI System Synthesis

Udacity mentor

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Overview and Industry Context

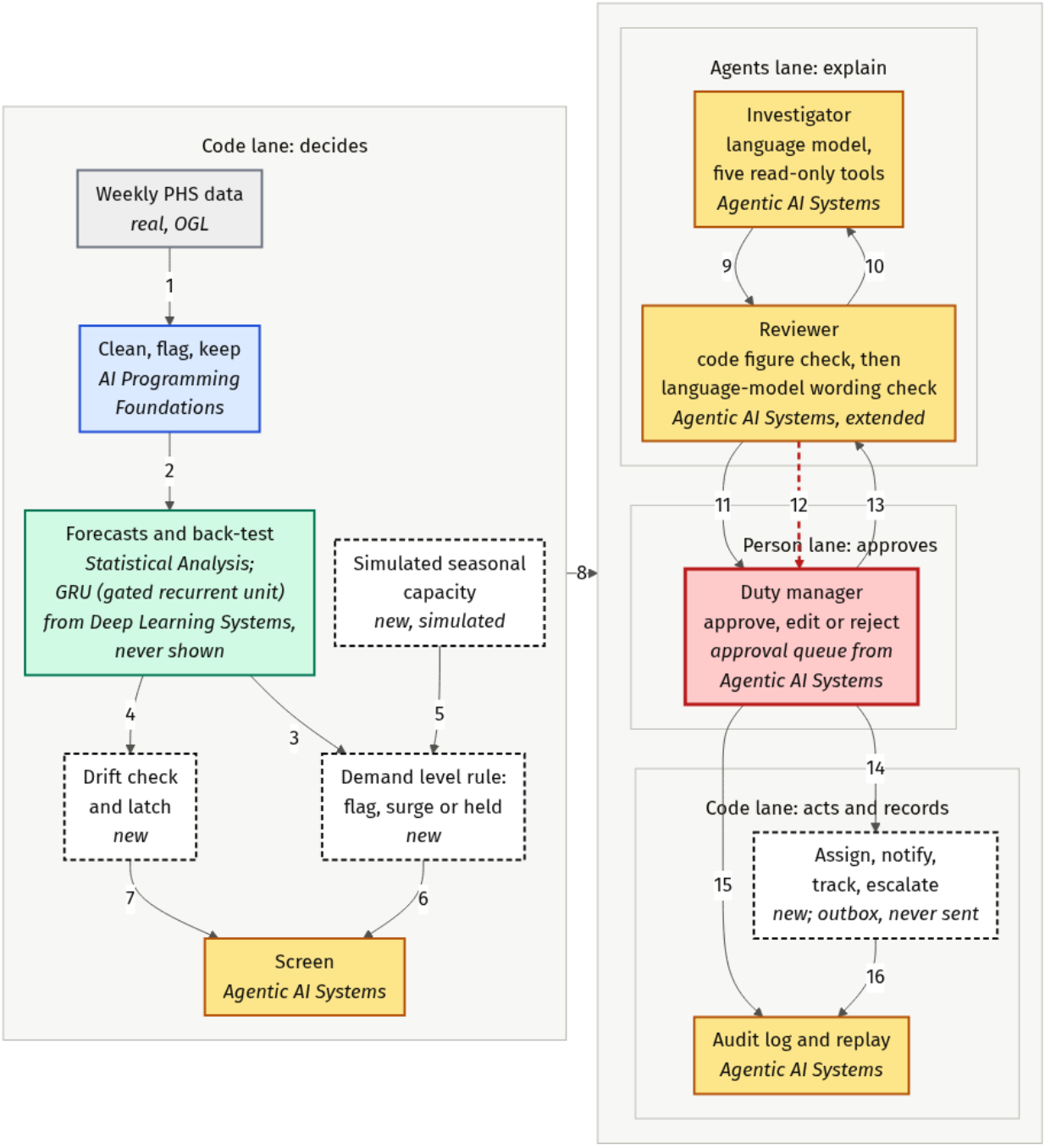
Industry: healthcare operations, unplanned attendances at Scotland's emergency departments (EDs), for the duty managers who staff the front door. Rotas are set weeks ahead: England's National Health Service (NHS) rota code gives resident doctors their rosters at least 6 weeks before a rotation (NHS England, 2025). ED crowding is linked with higher morbidity and mortality (Scottish Government, 2016). A forecast one to four weeks ahead can still inform extra shifts, cover from the staff bank or held leave.

At this stage the system gives the duty manager additional information beside their existing process. It replaces no current process and sets no staffing on its own. It needs shadow mode, a run on new weeks with nothing acted on, a pilot with managers who did not build it, and tuning, before it could be a primary operational tool.

Why artificial intelligence (AI)? Simple models forecast demand from open data on accident and emergency (A&E) attendances (Public Health Scotland [PHS], 2026), used under the Open Government Licence (OGL). A gated recurrent unit (GRU), a small neural network, runs beside them only as a comparator (Petravić et al., 2025). A duty manager also needs each forecast explained, with its range and how far to trust it. A language model agent drafts that explanation from read-only tools (Yao et al., 2023), while code sets every number (Figure 1).

Figure 1

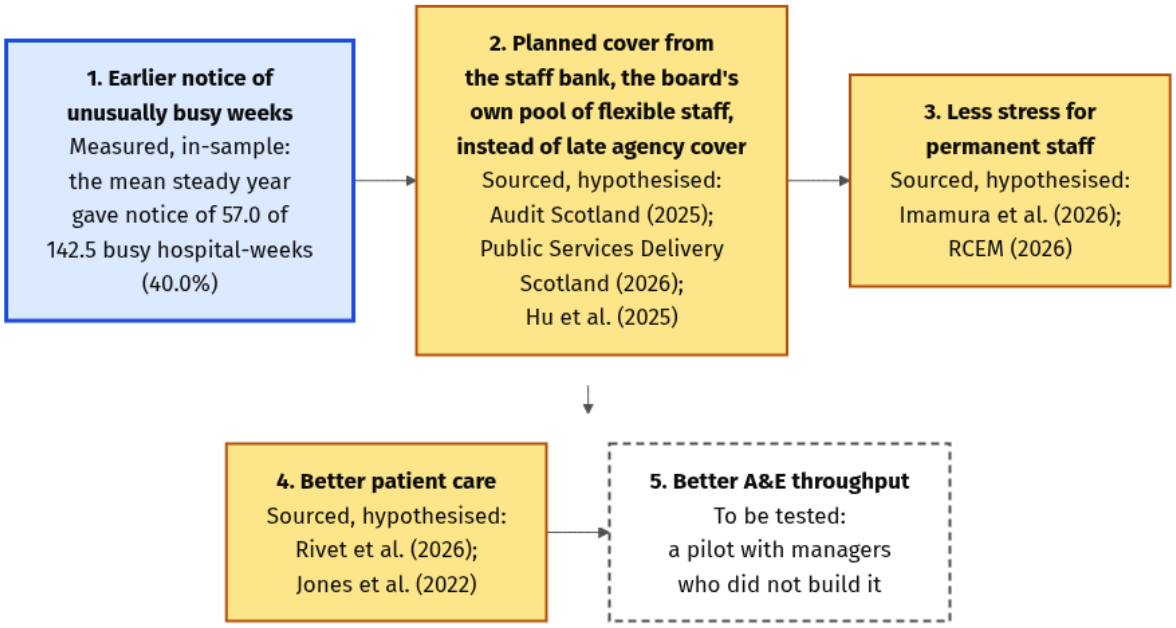
Code Decides, the Agents Explain, a Person Approves



In 2024/25 Scotland spent £256m on agency staff of all kinds and, separately, £353m on the nursing bank (Audit Scotland, 2025, p. 15). Any saving would be mainly the agency premium, with wider value for staff and patients. Figure 2 shows the value chain.

Figure 2

The Value Chain, Each Link Labelled by Its Evidence



Note. Busy means 1.05 times simulated capacity or more; the mean steady year averages 2019 and 2023 to 2025. Sources: Audit Scotland (2025), Public Services Delivery Scotland (2026), Hu et al. (2025), Imamura et al. (2026), Royal College of Emergency Medicine (2026), shown as RCEM, Rivet et al. (2026) and Jones et al. (2022).

Integration Rationale

I integrated four prior capstone projects (Table 1), spanning data analysis, statistics, deep learning and agentic systems, whose language model brings in generative AI. A fifth, Applied Machine Learning, shaped two decisions: results per site and the pilot.

AI Programming Foundations kept one unexplained merchant's payments, flagged, and the answer changed without them; so the COVID-19 weeks stay in, flagged, as a real shock. In Conduct a Statistical Analysis Using Python ("Statistical Analysis") the changes' standard deviation, 18.63 points, was below either period's spread (22.36 and 21.29), so pairing gave a more precise test. Here a paired interval compares the GRU with the primary on the same weeks. In Deep Learning Systems the best naive forecast placed the day's peak better than either network, so simple models stay the bar to beat. In Agentic AI Systems the Investigator cited figures no tool had returned, and the check caught one in ten; so here code writes every figure and a person approves every brief.

On five replay series at four horizons, outside weeks when a series' forecasts had stopped fitting, the paired interval favoured the primary in 16 of 20 comparisons and the GRU in none. The frontier model, hosted by Anthropic, adds one new technology, its client library, behind Agentic AI Systems' model gateway, chosen by a rule stated before the runs.

Table 1

Prior Capstone Projects and What Each Supplies

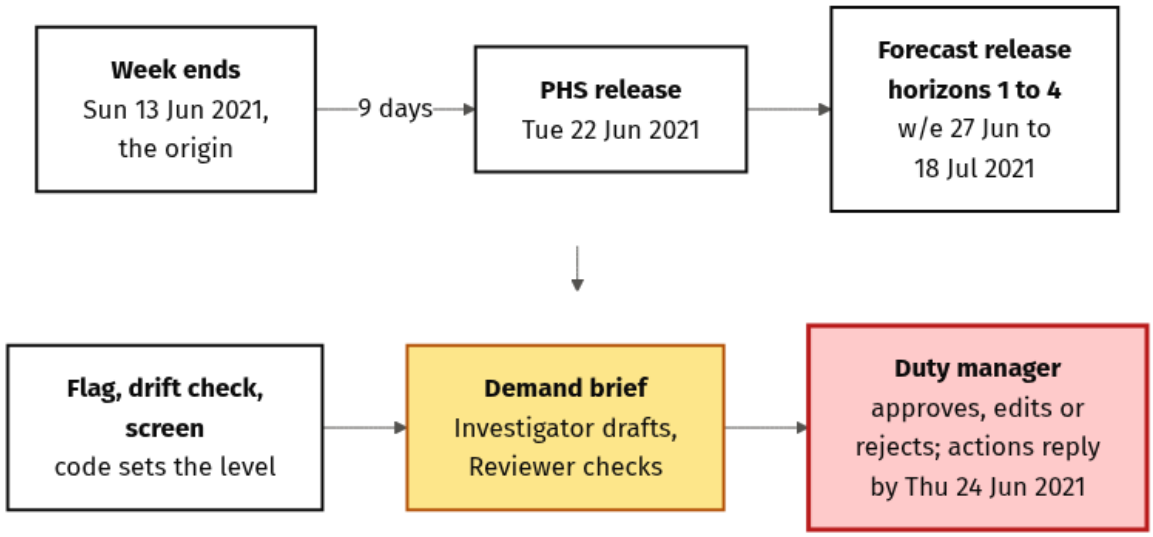
Prior capstone project	What it supplies here
AI Programming Foundations	The weekly load, the COVID-19 weeks kept and flagged
Statistical Analysis	The back-test and bootstrap (Hyndman & Athanasopoulos, 2021, Section 12.5) and the paired test of the GRU
Deep Learning Systems	Seasonal naive, the 4-week moving average and the GRU comparator
Agentic AI Systems	The screen, read-only tools, the figure check, replay, approval and audit

System Design and Technical Decisions

Figure 1 shows three lanes: code decides and then acts, the agents explain, and the duty manager approves, edits or rejects each brief. The data are weekly unplanned A&E attendances at 30 hospitals and nationally, 593 weeks to the week ending (w/e) 11 October 2026. Each week is released 9 days after it ends, so the forecasts cover the four weeks after each release, its release horizons (Figure 3).

Figure 3

One Week, From the Tuesday Release to the Duty Manager's Decision



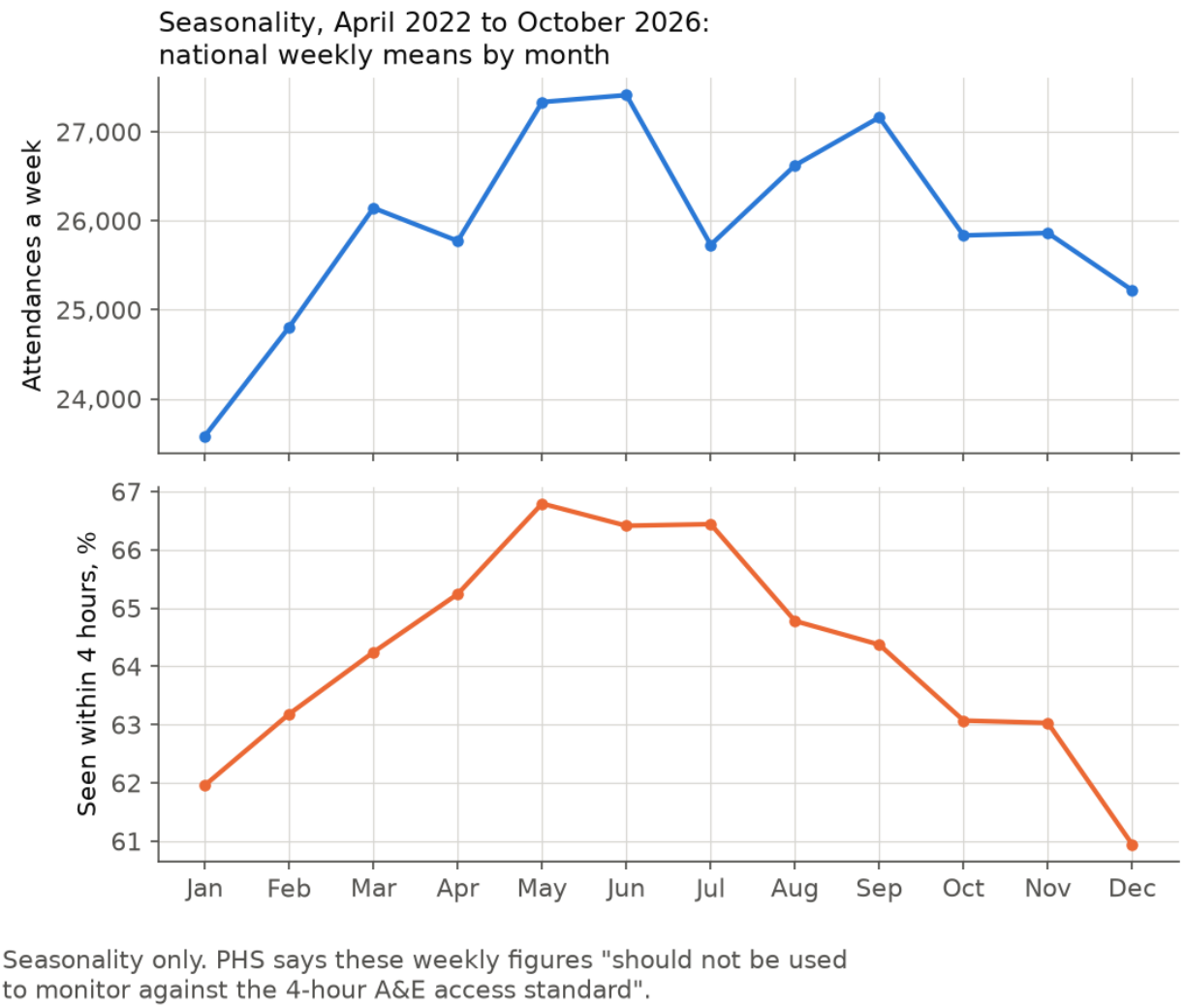
Note. Dates from Ninewells' demand week ending 13 June 2021, whose brief was rejected.

Four simple models forecast each series, among them seasonal naive, the same week last year, a standard benchmark (Hyndman & Athanasopoulos, 2021, Section 5.2). The open data hold no beds or staffing, so I use simulated capacity, a seasonal baseline of past demand: the 90th percentile of the hospital's attendances in the same weeks of the previous three years. Code flags an outlook, one hospital's next four weeks, when the primary model's last 104 errors give

any week at least a 50% chance of reaching 1.05 times capacity. The demand level shows demand unusually high for the time of year; it is not pressure, need or priority. It runs from 1 to 4; a flag raises a week to level 2. Its bands borrow only the structure of England's Operational Pressures Escalation Levels (OPEL) framework (NHS England, 2024). Figure 4 shows the level tracks attendances, not 4-hour performance.

Figure 4

Monthly Attendances Against 4-Hour Performance



Note. Data from PHS (2026), the source of the quotation in the image.

The drift alarm fires when attendances fall outside the one-week forecast plus or minus 1.96 standard deviations of its recent errors (Hyndman & Athanasopoulos, 2021, Section 5.5) for two released weeks running. It sets the drift latch, a switch that marks the hospital's forecasts untrusted and stops its flag until reset. A surge, an upward breach, takes the latest released week's level. In the design only a person resets the latch; this build simulates each reset. The primary model is then re-selected by its rolling-origin error on the latest 104 released weeks (Hyndman & Athanasopoulos, 2021, Section 5.10). Table 2 gives four tradeoffs.

Table 2

Four Tradeoffs That Shaped the Design

Choice	Gain	Cost
A simple primary model; the GRU only a comparator	A forecast checkable in one sentence	Any pattern the simple models miss
Capacity simulated from past demand	A level tested on real attendances	Illustrative, not about beds
A rare flag	In-sample, steady years: 53.8% of flags borne out, against 21.2% without one	Only 21.0% of busy outlooks flagged
Hosted Claude Sonnet 5.5, switchable to a local model in Ollama	5 of 7 briefs verified, against none locally	Cost per call, an outside service, no pinned version

The Investigator reasons between tool calls in the ReAct pattern (Yao et al., 2023), writing each figure as a placeholder that code fills. The Reviewer checks every figure against its tool result and screens for the words the Medicines and Healthcare products Regulatory Agency (MHRA) lists as indicative of a medical device (MHRA, 2023, p. 20). A language model then checks the wording. A brief that fails twice is marked "not verified, do not act" and can never be approved. After approval, code sets staffing actions, owners and deadlines, and a language model

words each notification, kept in an outbox, never sent. Each brief opens with what a duty manager needs at a glance and what to do, then the evidence (Figure 5).

Figure 5

A Brief as the Duty Manager Receives It

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Royal Infirmary of Edinburgh at Little France, week ending 23 Aug 2026 (figures released
  Tue 01 Sep 2026)

At a glance
- Level 2 of 4: demand may be higher than usual for the time of year from w/e 06 Sep
  2026 to w/e 27 Sep 2026. The level does not measure pressure, need or priority.
- Why: at least one of the next four weeks may run 5% or more above this hospital's
  high-week mark for the time of year. That mark is the level 9 weeks in 10 stayed under
  at this time of year over the last three years.
- Expected attendances: about 2,460 to 2,660 a week over those four weeks.
- The forecasts: the four methods' central figures are within 9% of each other.
- How accurate they have been: on past weeks these forecasts were out by 2.9% to 4.5% on
  average, for the first of the four weeks.
- Past flags here: when this hospital was flagged before, demand did turn out high, a
  week 5% or more above the mark, 4 times out of 5.
- No warning that the forecasts have stopped matching recent weeks.

What to do, if you approve
- Duty manager: Check whether extra shifts or bank staff are needed for w/e 06 Sep 2026
  to w/e 27 Sep 2026.
- Rota coordinator: Hold new leave requests for w/e 06 Sep 2026 to w/e 27 Sep 2026 until
  the duty manager's check is done.

The detail follows: each forecast with its ranges, past forecast errors and the track
  record (examples/current_brief_edinburgh.md).
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Note. The top of the Royal Infirmary of Edinburgh's brief for w/e 23 August 2026, the latest flag with no drift alarm, written by code; w/e 20 September 2026 did run 5% above its high-week mark. The full brief and other examples are in the repository's examples/ folder.

Ethical Considerations and Responsible AI

Automation bias and transparency. Agentic AI Systems named automation bias, here a manager approving a brief without weighing it (Goddard et al., 2012), as the next risk at its approval step. Against it, every brief opens with what the level means, the expected range, the forecasts' spread and past accuracy, or why none can be trusted, and the hospital's track record.

The approver, the duty manager, judges each against six criteria written in advance. A person can override every output, following human oversight as a principle, not a legal duty (European Union, 2024, Article 14).

Misuse and status as additional information. As Statistical Analysis warned its counts could be misused to name code breakers, this tool is never for ranking hospitals, justifying cuts or monitoring the 4-hour standard. It never sees patient data. Whether it is a medical device rests on my reading of the MHRA's guidance, so it stays an open risk (MHRA, 2023).

Bias by hospital size and accountability. Seasonal naive's median error is 11.0% at the smallest sites and 5.4% at the largest. Raised shares by site ran from 35.7% of outlooks at the Royal Hospital for Children to none at Glasgow Royal Infirmary, both in the largest size band, so the burden varies by site more than size. As Applied Machine Learning reported results per public body, each brief shows its own site's error, and the level is never read as need. Owners are by role, on the structure of OPEL's actions by level (NHS England, 2024), and a clinical safety reviewer would sign off before any live use.

Evaluation and Reflection

Every figure is in-sample: the design was tuned on the data that scores it. Error is the mean absolute percentage error (MAPE), as Hyndman and Athanasopoulos (2021, Section 5.8) define it. Figure 6 scores the eight goals, set in advance.

Figure 6

Goals G1 to G8, Each Against Its Threshold

Goal	Met if	Result	Verdict
G1 Forecasts a manager can plan with	within 1 point at every release horizon	-0.18 to -0.74 points against the best simple model	**met**
G2 Honest uncertainty	90% to 98%	95.7% to 96.4% of actuals inside	**met**
G3 Shocks are caught	by Tue 31 Mar 2020; at most 1 a year	known Tue 31 Mar 2020; at most 1 alarm a year	**met**
G4 No unchecked number approved	every approved brief passes; no not-verified brief approved; plant caught	5 of 5 approved pass; 2 not verified, neither approved; plant caught	**met**
G5 The agent never sets a level	always the rule's level	7 of 7 briefs written	**holds**
G6 Briefs are useful to a person	6 of the 7 briefs approved with no factual change, and Reader B makes the approver's decision on at least 3 of the 4 they read	5 of 7 (the two demand-week briefs were refused and rejected); Reader B agreed on 4 of 4	**not met**
G7 Raised levels rare in steady years	under 10% in every steady year	over 10% in 3 years, 10.4% to 11.7%	**narrowly missed**
G8 A flag tells a manager something	at least twice the base rate	2.54 times the base rate, steady years	**met**

G1 to G3, G7 and G8 are in-sample, on 2015 to 2026. G4 to G6 are on the frontier set's seven replay briefs. G5 is an invariant, not a test.

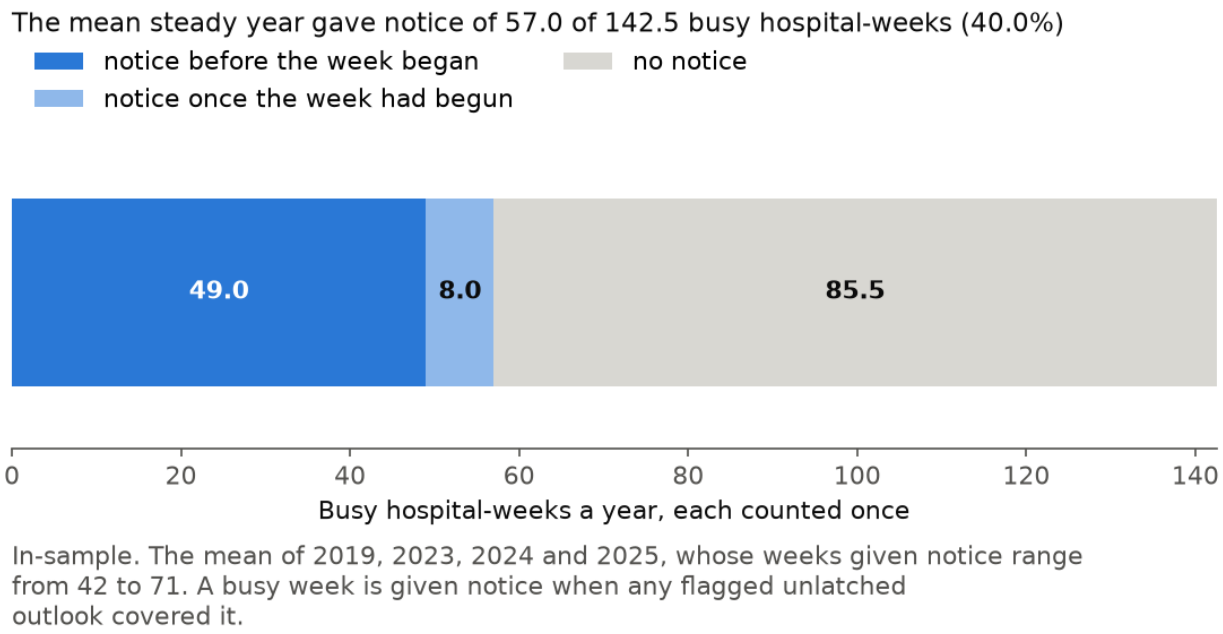
Note. Reader B: a second reader who did not build the system.

What worked. Five goals are met: "forecasts a manager can plan with" (G1), "honest uncertainty" (G2), "shocks are caught" (G3), "a flag tells a manager something" (G8) and "no

unchecked number reaches a manager as approved" (G4). Outside shock weeks, the primary model's national MAPE is within 1 point of the best simple model at every release horizon, and measurably lower only at the fourth. In the mean steady year the flag alone gave notice of 57.0 of 142.5 busy hospital-weeks (Figure 7). "The agent never sets a level" (G5), an invariant, holds.

Figure 7

Busy Hospital-Weeks Given Notice in the Mean Steady Year



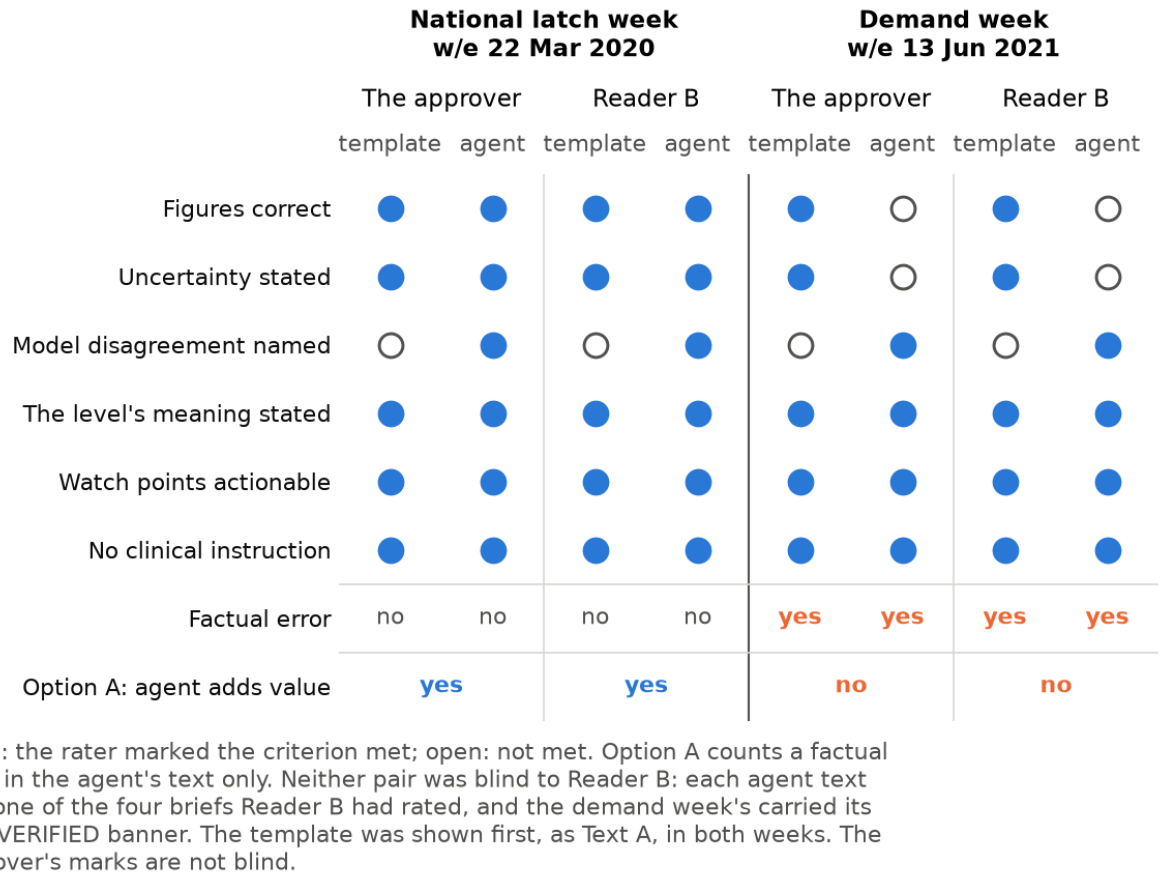
In-sample. The mean of 2019, 2023, 2024 and 2025, whose weeks given notice range from 42 to 71. A busy week is given notice when any flagged unlatched outlook covered it.

What did not work. The adequacy test, a pass mark stated before any brief was written, needed 6 of the 7 replay briefs verified and meeting at least 5 of the 6 criteria. The frontier model came closest, at 5 of 7, and the approver chose its briefs. "Briefs are useful to a person" (G6) had two tests. First, 6 of the 7 briefs had to be approved with no factual change; 5 were, as the two demand-week briefs were refused and rejected. Second, Reader B had to make the approver's decision on at least 3 of 4 briefs, and did on all 4. So G6 is not met; G7 is narrowly missed (Table 3, which lists each failure).

The template comparison, judged by a rule fixed before the ratings (Figure 8), found the agent's explanation added value in the national latch week for both raters.

Figure 8

Template Against Agent: Both Raters, Both Weeks



Note. Option A: the agent's text adds value in a week only if it meets every criterion the template's text meets, and at least one more, with no factual error. Raters marked the demand-week agent text's figures wrong because of its NOT VERIFIED banner, though they passed the code check.

Both counted it as naming the models' disagreement, because it marked every model untrusted while latched and ranked none; the template showed differing forecasts without saying

none should be preferred. In the demand week it did not: that text was the refused brief, whose trend and fit claims the checks had stopped. So it adds value overall for neither rater.

Table 3

Failure Cases, With Result and Cause

Run or case	Result	Cause
Local model, round 1	3 of 7 completed before its 840-second budget stopped it; the code check refused every draft, so none was written or verified	A fault in my code that calls the model sent each retry blind, without the first attempt's problems; fixed before round 2
Local model, round 2	7 of 7 completed; none written or verified; a retry with thinking off is untried	Every draft was cut at the 8,192-token output cap, thinking included
Frontier model, round 2	5 of 7 verified; both demand-week briefs refused, then rejected; the template now writes demand weeks	Ninewells' claimed a rising trend from one figure, Lorn & Islands' a flat forecast from one; both echoed code's fit line
"Raised levels are rare in steady years" (G7), under 10%	10.4% to 11.7% in 2018, 2023 and 2024; by design no raised level sets staffing without a manager's approval	My untested reading: demand outgrew the years that set capacity; shadow mode tests it
COVID-19, March 2020	Every model missed, one by 152.8%; the latch kept forecasts untrusted	A shock breaks any model trained on past weeks; shadow mode tests the latch
No notice of 85.5 of 142.5 busy hospital-weeks in the mean steady year	By design the system only adds information, so those weeks rest on the existing process, as before	A rare flag trades recall for precision; the pilot learns what planners use

Note. Replay briefs are seven real past weeks; the notebook and case study give each in full.

By that rule, the template now explains demand weeks by default, and the agent latch, surge and model-disagreement weeks. Model disagreement has no threshold yet, so it keeps no week; setting one is a next step. New live briefs follow it; template briefs are queued for approval like the agent's. The comparison keeps version 1; the shipped version drops the drift

line both raters marked wrong and opens in plain words with what to do. This came after the ratings, so the pilot tests it.

An AI independent judge read the seven briefs first and drafted the five edits the approver adopted, none of them factual. An AI reviewer, not the system's Reviewer, scored the adequacy test. I built the system and played the approver, which the second reader exists to check.

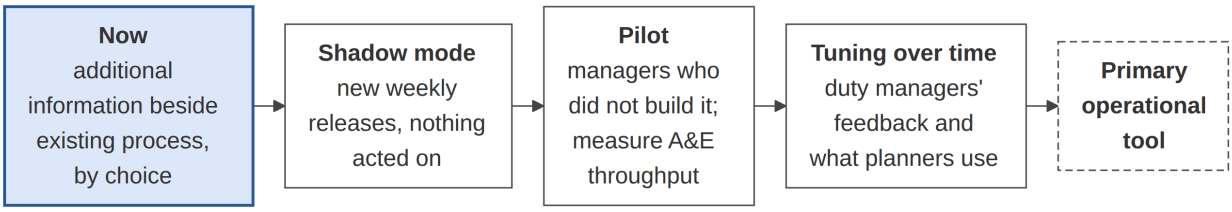
What I learned. Across performance, complexity and responsible use, the GRU's refit and seeds bought no measurable gain, and the frontier model's 5 of 7 verified briefs cost money per call and an outside service. Integration's value lay in the checks between parts, which stopped both unsupported briefs before approval. The agent explained latch and surge weeks well enough to approve, but overreached in both demand weeks, where it had to explain high demand.

Professional Relevance

I kept this a pilot build on purpose, as Applied Machine Learning's first release left its payback for a pilot to show; Figure 9 shows the path to a primary operational tool.

Figure 9

From the Pilot Build to a Primary Operational Tool



This work prepares me for AI solutions architect and machine learning engineer roles, drawing on my enterprise resource planning (ERP) work, where people approve a forecast before anyone acts. Its habits, tests stated in advance, a person at every decision and each failure reported with its cause, are what I would bring to an NHS AI team.

References

Audit Scotland. (2025, December 4). *NHS in Scotland 2025: Finance and performance*. https://audit.scot/uploads/2025-11/nr_251204_NHS_in_Scotland_2025.pdf

European Union. (2024). Regulation (EU) 2024/1689 of the European Parliament and of the Council of 13 June 2024 laying down harmonised rules on artificial intelligence (Artificial Intelligence Act), as amended by Regulation (EU) 2026/1744. *Official Journal of the European Union*, L, 2024/1689. http://data.europa.eu/eli/reg/2024/1689/oj

Goddard, K., Roudsari, A., & Wyatt, J. C. (2012). Automation bias: A systematic review of frequency, effect mediators, and mitigators. *Journal of the American Medical Informatics Association, 19*(1), 121-127. https://doi.org/10.1136/amiajnl-2011-000089

Hu, Y., Chan, C. W., Dong, J., Kazekjian, A., Ophaswongse, C., Sugalski, G., Underwood, J. P., & Perotte, R. (2025). Implementing a prediction driven framework for emergency department nurse staffing to optimize real time decisions. *npj Health Systems, 2*(1), Article 16. https://doi.org/10.1038/s44401-025-00019-2

Hyndman, R. J., & Athanasopoulos, G. (2021). *Forecasting: Principles and practice* (3rd ed.). OTexts. https://otexts.com/fpp3/

Imamura, Y., Sakuramoto, H., Ouchi, A., Honda, T., Uno, S., & Tamoto, M. (2026). Emergency nurse staffing and outcomes for patients and nurses: A scoping review. *Journal of Emergency Nursing*. Advance online publication. https://doi.org/10.1016/j.jen.2026.05.015

Jones, S., Moulton, C., Swift, S., Molyneux, P., Black, S., Mason, N., Oakley, R., & Mann, C. (2022). Association between delays to patient admission from the emergency department

and all-cause 30-day mortality. *Emergency Medicine Journal, 39*(3), 168-173. https://doi.org/10.1136/emermed-2021-211572

Medicines and Healthcare products Regulatory Agency. (2023). *Medical device stand-alone software including apps (including IVDMDs)* (Version 1.10g; file revised October 2026). GOV.UK. Retrieved October 24, 2026, from https://assets.publishing.service.gov.uk/media/6abf6c8c943e890cb55e5e52/Medical_device_stand-alone_software_including_apps__including_IVDMDs.pdf

NHS England. (2024, December 2). *Integrated operational pressures escalation levels (OPEL) framework 2024 to 2026* (page updated May 23, 2025). https://www.england.nhs.uk/publication/integrated-opel-framework-2024-to-2026/

NHS England. (2025). *Resident Doctors (RDs): Rota Code of Practice compliance data collection* (DCB3042 AMD 35/2025) [Data collection]. https://standards.nhs.uk/published-standards/resident-doctors-rds-rota-code-of-practice-compliance-data-collection

Petravić, L., Gril Rogina, K., Albreht, T., Kukec, A., & Žibert, J. (2025). Prognostic models for predicting patient arrivals in emergency departments: An updated systematic review and research agenda. *BMC Emergency Medicine, 25*(1), Article 106. https://doi.org/10.1186/s12873-025-01250-8

Public Health Scotland. (2026). *Weekly A&E activity and waiting times* [Data set]. Scottish Health and Social Care Open Data. Retrieved October 24, 2026, from https://www.opendata.nhs.scot/dataset/weekly-accident-and-emergency-activity-and-waiting-times

Public Services Delivery Scotland. (2026, June 2). *NHSScotland workforce: Latest statistics at 31 March 2026* [Accredited official statistics]. Turas Data Intelligence. https://turasdata.nes.nhs.scot/data-and-reports/official-workforce-statistics/all-official-statistics-publications/02-june-2026-workforce/?pageid=16251

Rivet, C., Amazouz, H., Gratacap, M., Delannoy, Q., Yordanov, Y., Leblanc, J., & URGE collaborators. (2026). Association between patient-to-nurse ratios in emergency departments and patient, staff, and organizational outcomes: A systematic review. *European Journal of Emergency Medicine, 33*(4), 237-247. https://doi.org/10.1097/MEJ.0000000000001350

Royal College of Emergency Medicine. (2026). *The state of emergency medicine in Scotland* [Report]. https://rcem.ac.uk/wp-content/uploads/2026/03/RCEM_SoEM_Scotland_A4_0326.pdf

Scottish Government. (2016). *Emergency department capacity management: Guidance*. https://www.gov.scot/publications/emergency-department-capacity-management-guidance-2/

Yao, S., Zhao, J., Yu, D., Du, N., Shafran, I., Narasimhan, K., & Cao, Y. (2023). ReAct: Synergizing reasoning and acting in language models. In *The Eleventh International Conference on Learning Representations (ICLR 2023)*. https://openreview.net/forum?id=WE_vluYUL-X

Glossary: every term used in this paper is defined in glossary.pdf, submitted with it.